

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction for the administration of Mounjaro (tirzepatide) for the treatment of Obesity / Weight Management

Summary of NICE / NICE CKS Guidelines for Obesity (Weight Management in Adults)

Overview

- Obesity is defined as excess body fat that presents a risk to health.
- Diagnosis is based on Body Mass Index (BMI), with waist circumference used to assess central fat distribution (especially in people with BMI <35).

Classification (Adults)

BMI (kg/m²)	Classification
18.5–24.9	Healthy weight
25–29.9	Overweight
30–34.9	Obesity Class I
35–39.9	Obesity Class II
≥40	Obesity Class III (severe/morbid)

Adjust BMI thresholds for South Asian, Black African, and some other minority ethnic groups (e.g. action at BMI ≥23 for overweight, ≥27.5 for obesity).

Assessment

- Measure: Weight, height, BMI, and waist circumference.
- Assess comorbidities (type 2 diabetes, hypertension, sleep apnoea, CVD), lifestyle factors (diet, activity, sleep, smoking, alcohol), readiness to change and previous weight loss efforts, and medications contributing to weight gain.

Management Approach (Stepwise)

1. Lifestyle Intervention (First-Line for All)

- Multicomponent programme: calorie deficit (~600 kcal/day deficit or 1200–1500 kcal/day), ≥150 minutes/week moderate-intensity activity, behavioural strategies (goal-setting, self-monitoring, problem-solving).
- Group-based programmes are often more effective.

2. Pharmacological Treatment

- Consider only if BMI ≥ 30 (or ≥ 27 with comorbidities) and lifestyle measures have failed.
- This PGD covers Mounjaro for weight management; see separate PGDs for other pharmacological options where authorised.
- Monitor side effects and response. Reassess at defined milestones.

3. Specialist Referral / Tier 3 Services

- Refer if BMI ≥ 40 , or ≥ 35 with comorbidities; or failure of Tier 2 interventions; or consideration of additional pharmacological or surgical options.

4. Bariatric Surgery (Tier 4)

- Consider for BMI ≥ 40 , or ≥ 35 with comorbidities, after failure of non-surgical options. Patient must be committed to long-term follow-up.

Follow-Up

- Regularly monitor weight, BMI, waist circumference, comorbidities.
- Support ongoing lifestyle changes.
- Relapse is common — plan for long-term support.

Patient Group Direction
for the supply/administration of
Mounjaro (tirzepatide) pre-filled pen
for the treatment of
Obesity / Weight Management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC. • Pharmacy technician registered and practicing with the GPhC.
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SmPC for the products as well as BNF advice and any applicable national guidelines. • All users must previously have used PGDs to supply medication. • Must work in compliance with the SOPs of their own employer. • All users must have appropriate indemnity in place to cover this service.
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal. • All users must be up to date with MHRA and safety alerts with regards to medical products. • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.
PGD Indication	Mounjaro (tirzepatide) is a dual GIP/GLP-1 receptor agonist indicated as an adjunct to a reduced-calorie diet and increased physical activity for weight management, including weight loss and weight maintenance, in adults with an initial Body Mass Index (BMI) of:

	• ≥ 30 kg/m² (obesity), or • ≥ 27 kg/m² to < 30 kg/m² (overweight) in the presence of at least one weight-related comorbidity (e.g. dysglycaemia [pre-diabetes or type 2 diabetes mellitus], hypertension, dyslipidaemia, obstructive sleep apnoea or cardiovascular disease).
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Initial Assessment

Before commencing treatment with weight management therapies, the patient must be assessed by a registered healthcare professional. The initial assessment should be face-to-face.

The assessment should include, but may not be limited to:

- Causes of weight gain — the patient should be referred to their GP if their prescribed medication is causing weight gain.
- Patient's lifestyle, diet and level of exercise.
- Has the patient tried to lose weight in the past? Discuss measures that may have been successful or unsuccessful.
- Explore other factors that may be causing weight gain, such as mental health, environmental and psychological factors. Consider signposting to another health professional if needed, e.g. a mental health team.
- Does the patient have other disease states that may pre-dispose them to weight gain? Consider referring to a GP for further advice.
- What are the patient's expectations in terms of weight loss? Are the expectations realistic for their height and body frame?
- Calculate Body Mass Index (BMI) and determine the patient's ideal weight. Set a realistic target weight with the patient and agree review intervals.

Inclusion criteria	• Adults aged 18 to 75 years (inclusive). • BMI ≥ 30 kg/m², or BMI ≥ 27 kg/m² with at least one weight-related comorbidity (e.g. hypertension, type 2 diabetes mellitus, dyslipidaemia, obstructive sleep apnoea, established cardiovascular disease). • Adjusted BMI thresholds applied for South Asian, Black African and some other minority ethnic groups (e.g. action at BMI ≥ 27.5 kg/m² for obesity). • Patient is willing to follow a reduced-calorie diet and increase physical activity in line with the agreed lifestyle plan. • Initial assessment completed and documented (see Initial Assessment section above). • Informed consent obtained (including off-label considerations where applicable, side-effect profile, and treatment expectations). • No exclusion criteria present.
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Exclusion criteria	• Adults under 18 years of age. • Adults aged over 75 years (upper age limit under this PGD; refer to specialist if treatment is being considered). • Pregnancy, breastfeeding, or planning pregnancy (effective contraception required; advise discontinuation at least 2 months before planned conception for Wegovy/semaglutide, 1 month for Mounjaro/tirzepatide). • Known hypersensitivity to the active substance or to any of the excipients. • Personal or family history of medullary thyroid carcinoma (MTC) or Multiple Endocrine Neoplasia syndrome type 2 (MEN 2). • History of pancreatitis (acute or chronic). • Severe gastrointestinal disease, including gastroparesis or severe persistent gastrointestinal disorder. • Concurrent use of any other GLP-1 receptor agonist or insulin secretagogue used for weight management. • Type 1 diabetes mellitus. • Diabetic retinopathy (treatment may worsen retinopathy; defer or refer to specialist). • Severe renal impairment (eGFR <30 mL/min/1.73 m²) or end-stage renal disease. • Severe hepatic impairment. • History of suicidal ideation or active severe mental illness without psychiatric oversight. • Active eating disorder (anorexia nervosa, bulimia, binge-eating disorder under specialist care). • BMI below the PGD inclusion threshold. • Patients who, in the clinical judgement of the healthcare professional, are not suitable for the medication.
Cautions	• Counsel on gradual dose escalation and gastrointestinal side effects; consider delaying titration or reducing to the previous dose if significant symptoms occur. • Gallbladder disease — increased risk; counsel on symptoms (RUQ pain, jaundice, fever) and refer urgently if suspected. • Mild–moderate renal impairment: monitor for dehydration secondary to GI side effects. • Concomitant oral medications: GLP-1 / GIP agents delay gastric emptying — counsel patients on possible reduced absorption of oral medications (especially

	narrow therapeutic index drugs and oral contraceptives — non-oral contraception or barrier method advised during titration and for 4 weeks after each dose increase). • Counsel on signs and symptoms of pancreatitis; discontinue and refer urgently if suspected. • Sulfonylurea or insulin co-use (in patients with comorbid T2DM) carries hypoglycaemia risk — refer to the prescribing GP/specialist before starting. • Risk of dehydration — counsel on adequate fluid intake during GI side effects. • Reassess benefit-risk if no clinically meaningful weight loss (typically <5%) after 6 months on maintenance dose. • Rotate injection sites to reduce local irritation.
Actions if patient is excluded or declines treatment	• Discuss the reason for exclusion with the patient and ensure they understand. • Advise on alternative treatment options and how these can be accessed (e.g. GP, specialist weight management service, lifestyle programmes). • If the exclusion relates to an undiagnosed or unmanaged comorbidity, recommend GP review. • Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Mounjaro (tirzepatide) solution for injection in pre-filled pen (KwikPen / single-dose pen depending on supplied product). Available strengths: 2.5 mg, 5 mg, 7.5 mg, 10 mg, 12.5 mg, 15 mg per 0.5 mL dose.
Legal category	POM
Storage	• Store in a refrigerator (2°C to 8°C). Do not freeze. If frozen, discard. • Once removed from refrigeration, may be stored unrefrigerated below 30°C for up to 21 days. Discard if not used within this period. • Keep the pen in the outer carton in order to protect from light. • Travel: when travelling by air, carry pens in hand luggage. Pens should not be placed in checked baggage where temperatures cannot be guaranteed. A travel letter from the prescriber/pharmacist may be

	required.
Route/method of administration	• Subcutaneous injection in the abdomen, thigh, or upper arm. Rotate injection sites. • Administered once weekly, on the same day each week, with or without food, at any time of day. • The day of weekly administration can be changed if necessary as long as the time between two doses is at least 3 days (>72 hours). After selecting a new dosing day, once-weekly dosing should be continued. • Do not administer intravenously or intramuscularly.
Dose and frequency	Starting dose: 2.5 mg once weekly for 4 weeks (titration dose; not intended for sustained weight management). Maintenance escalation: • After 4 weeks at 2.5 mg, increase to 5 mg once weekly. • Further dose increases of 2.5 mg increments may be made after a minimum of 4 weeks on the current dose if additional glycaemic / weight management is required and the current dose is tolerated. • Recommended maintenance doses: 5 mg, 10 mg or 15 mg once weekly. • Maximum dose: 15 mg once weekly. If a dose is missed, administer as soon as possible within 4 days (96 hours). If more than 4 days have passed, skip the missed dose and resume usual schedule. Do not administer two doses within 3 days. If significant gastrointestinal symptoms occur, consider delaying dose escalation or reducing to the previous dose until symptoms have improved.
Quantity to be administered and/or supplied	• One pre-filled pen contains one weekly dose at the prescribed strength. • Supply 4 pens (one month's treatment) per patient appointment. • If the patient is going on holiday, their appointment may be brought forward by a few days to allow collection of medication, and to ensure continuity of treatment. • This PGD does not allow additional medication to be supplied to enable patients to 'stock up'.
Maximum or minimum treatment period	Provided that the patient is assessed before each supply is made and continues to meet eligibility, there is no fixed maximum treatment period. • Reassess clinical benefit, tolerability and target weight

	at each visit. • If the patient has not lost at least 5% of their initial body weight after 6 months on the maximum tolerated dose, a decision is required on whether to continue treatment, taking into account the benefit-risk profile in the individual patient. • When the patient reaches their target weight, discuss with them whether to continue treatment to maintain their weight. • If the patient has used Mounjaro in the past and wants to recommence treatment, the dose must be titrated again following the schedule above. The BMI inclusion criteria for initiation must be reapplied if more than 2 months have passed since discontinuing treatment.
Adverse effects	Very common ($\geq 1/10$): • Gastrointestinal: nausea, diarrhoea, vomiting, constipation, decreased appetite. Common ($\geq 1/100$ to $< 1/10$): • Abdominal pain, dyspepsia, eructation, flatulence, abdominal distension, GORD. • Injection-site reactions (redness, pruritus). • Hypoglycaemia (when used with sulfonylurea or insulin). • Fatigue, dizziness, hair loss. • Tachycardia. Uncommon and rare: • Acute pancreatitis. • Cholelithiasis, cholecystitis. • Hypersensitivity reactions including anaphylaxis (rare). • Acute kidney injury (typically secondary to dehydration). Counsel patient on warning symptoms (especially severe abdominal pain, severe vomiting and dehydration, jaundice). Refer to current SmPC for full safety information.
Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • That valid informed consent was given. • Name of individual, address, date of birth and GP.

	• Name of healthcare practitioner. • Name and brand of medication. • Date of supply, dose, form, route and quantity supplied. • Advice given, including advice given if excluded or declines treatment. • Details of any adverse drug reactions and actions taken. Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over — keep records for 8 years. • Children aged under 18 years — keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday).
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Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication. Provide written information on injection technique, missed-dose rules, sharps disposal, side-effect counselling and emergency contact details.
Follow-up advice to be given to patient or carer	• Seek medical advice if symptoms worsen rapidly or significantly, the patient is unable to tolerate the medication, or they become systemically very unwell. • Adhere to the agreed lifestyle changes — diet, physical activity, sleep, alcohol and smoking — alongside the medication. • Inject as instructed, rotating injection sites, and report any persistent or worsening injection-site reactions. • Report and seek urgent medical advice for severe abdominal pain (possible pancreatitis), persistent vomiting/dehydration, signs of gallbladder disease (RUQ pain, jaundice, fever), or symptoms suggestive of severe allergic reaction. • Attend all review appointments. Bring weight diary / measurements if requested. • Do not share pens. Dispose of used needles in an approved sharps container.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017. <https://www.nice.org.uk/guidance/mpg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017.
- NICE CKS Guidance: Obesity.
- British National Formulary (BNF).
- Summary of Product Characteristics (SmPC) for the medicinal product.

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates

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Name of healthcare professional	Job title	Registration number	Signature

Valid from date

Expiry date

12/05/26	11/05/27

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026
Name of professional group signatory	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		12/05/26
Chris Pilkington	Pharmacist GPhC: 2046322		12/05/26
Janey Tipping	Pharmacist (Clinical Lead) GPhC: 2049827		12/05/26

Change history

Version number	Change details	Date
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001	Development and issue of standalone Mounjaro PGD (split from combined Weight Management PGD; new Initial Assessment section added; upper age limit 75 years; 2-month re-initiation BMI rule applied per clinical lead decision).	12th May 2026